



OFFICIAL

Fall and Fall Injury Prevention and Management Policy

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1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	All disciplines

2. Purpose

A fall is an unplanned event which results in a person coming to rest inadvertently on the ground or lower level¹⁻³. Falls and falls-related injuries cause substantial morbidity and mortality among older Australians. For those over 65, falls are a leading cause of injury related hospitalisation in WA¹⁻⁴.

The purpose of this document is to establish a uniform approach to fall injury prevention and management that is consistent with Australian best practice guidelines and the requirements of the National Safety and Quality Health Service Standards.

3. General principles / Information

Armada Kalamunda Group (AKG) will respond to risk of falls and harm from falls in ways that respect the individual's rights, dignity, autonomy, and decision-making capacity and use evidence-based, multidisciplinary team care.

- Falls are one of the most common adverse events experienced in hospitals⁵.
- Once a person has had one fall, they are at risk of having more falls.
- All falls are to be treated seriously by staff as often a fall is an indication of an underlying problem that can be treated
- Even relatively minor falls can lead to significant injury and death⁵.
- Patients who have had a fall and are seriously injured during their hospital stay, increase their length of stay by more than six days⁶.
- 80-90% of falls in hospital are unwitnessed.
- 50-70% occur by the bedside (from bed, bedside chair or transferring between them)
- More than half of inpatient falls occur for patients with a known cognitive impairment (such as delirium / dementia)⁷.
- 10-12% of falls occur in the toilet/ bathroom.

Patients are to have a comprehensive and multifactorial approach to falls prevention, involving the use of risk assessment and targeted interventions that address the individuals risk factors and clinical needs.

4. Falls Screening Assessment

4.1 Screening and assessment

There are multiple factors that indicate a patient is a falls risk. Utilise the table in [Appendix 1](#) and clinical judgement to identify if a patient is a falls risk. All patients admitted to the hospital should be screened for fall risk as soon as practicable (and within eight hours of admission). Document and action the appropriate interventions to minimise that risk within the following:

- Adult Triage Nursing Assessment (AKMR1.0.1).
- Falls Risk Assessment and Management Plan (AKMR24) (in some clinical areas will be on pause with the comprehensive care plan rollout).
- Falls Risk Assessment Medication Evaluation (AKMR 24.2) (in some clinical areas will be on pause with the comprehensive care plan rollout).
- Paediatric Falls and Pressure Injury Risk Assessment (AKMR115.2).
- Clinical Risk Management Tool Dialysis (AKMR24.1).
- Comprehensive Care Plan in areas where in place.

Staff should use interpreters and other communication strategies during falls risk assessment and management – see [Interpreter and Translator Guideline](#) for further information

4.2 Cognitive impairment

- More than half of inpatient falls occur for patients with known cognitive impairment (CI), including dementia and delirium).
- Possible communication difficulties experienced by people with CI can make subjective assessments unreliable. Special attention needs to be given to altered behaviours and nonverbal cues in this population.
- CI can increase the risk of falling by directly influencing the older person's ability to understand and manage environmental hazards, through a tendency to increased wandering, and through altered gait patterns and impaired postural stability/balance, difficulty or inability to recall education to use nursing call bell prior to mobilising.

Good practice points:

- Patients presenting to a hospital with an acute change in cognitive function should be assessed for delirium (via 4AT) and the underlying cause of this change. Refer to the [AKG Delirium Clinical Management Guideline](#).
- If a patient with CI does fall, reassess their cognitive status, including presence of delirium (via 4AT).
- Where possible and appropriate, involve family and carers in decisions about how best to support the patient whilst in hospital. Completing **The Top 5: What Matters Most** will assist delivery of personalised care.
- Interventions for people with confirmed or suspected CI may need to be modified and supervised, as appropriate.
- Patients with CI may benefit from scheduled toileting and being prompted to void⁹.

4.3 Feet and footwear

The presence of foot problems, such as pain, toe deformities, toe muscle weakness and reduced ankle flexibility, can alter the pressure distribution beneath the feet and impair balance and functional ability. Additionally, these foot problems are associated with an increased risk of falls, and the risk rises with the number of foot problems.

Walking barefoot or in socks is associated with a 10–13x increased risk of falling.

An assessment of footwear and foot problems should be included as part of an individualised, multifactorial, and multiple intervention approach for preventing falls.

Inappropriate or poor fitting footwear increases falls and fractures in older people.

4.4 Medications

Several factors affect an older person's ability to deal with and respond to medications, which can increase their risk of falls. This includes: ¹²

- Polypharmacy.
- The ageing process, as well as disease states, can result in changes to medication pharmacokinetics (absorption, distribution, metabolism and excretion of drugs) and pharmacodynamics (effect of drugs on cellular and organ function).

Additionally, medication non-adherence (including misuse and overuse) or inappropriate prescribing can worsen adverse effects, contributing to increased falls risk.

Certain classes of medication increase the risk of falls in older people, such as centrally acting or psychoactive medications (e.g. Benzodiazepines, antidepressants), anti-epileptics and antihypertensives.¹²⁻¹⁴

Refer to the FRAME tool (AKMR24.2) (in some clinical areas will be on pause with the comprehensive care plan rollout) [Appendix 2](#) for a list of medications that may increase the risk of falls and [Appendix 2](#) for further details on the FRAME.

4.5 Continence

Incontinence can increase a person's falls risk; incontinence should be routinely screened for.

In some cultures, incontinence is a taboo topic. Specific information on dealing with these issues may be obtained from the person, their carers, or the Continence Foundation of Australia.

Incontinence is not a condition that is well understood by Aboriginal Australians, and it causes shame for many. When discussing incontinence, it is important to be aware that Aboriginal men will frequently discuss this matter only with a male health worker and women only with a female health worker.

Special considerations: Maternity patients

- Any pre-existing conditions which may increase the woman's risk of falling during her hospital admission should be identified at her antenatal booking appointment.
- Well women who have an intervention, such as epidural analgesia or Caesarean section will temporarily have an increased risk of falling.
- Women will have their risk of falls assessed and recorded on the postnatal care plan.

- Women who have their risk of falls temporarily increased will need to have their motor function reassessed regularly by means of a Bromage score before ambulation is attempted.

Clinical staff are to document the planned evaluation of fall prevention interventions in the patient's healthcare record using the Digital Medical Record (DMR) Progress Notes and include any rationale for changes to interventions, where applicable.

4.6 Risk Re-Assessment

Once the patient has been admitted, a Risk Re-Assessment can be completed by any member of the MDT. It can be completed by a single member or in collaboration with other members of the MDT (e.g nurse & physiotherapist). A risk re-assessment **must** be completed on:

- Ward transfer.
- Family/patient request.
- New risk identified.
- Post incident i.e. fall.
- Change in condition.
- Previous Patient Care Plan full.
- This includes identification of risk factors which may have changed, effectiveness of existing falls prevention strategies and any additional strategies that may be warranted.
- If existing strategies are effective, then continue to put these in place.
- If strategies are ineffective consider other strategies.
- It is recommended that you discuss with a senior staff member and/or other members of the multidisciplinary team the falls prevention plan for the patient as there may be other strategies that have not been considered.

5. Engaging patients and carers

Falls prevention is best managed utilising a multifactorial collaborative care approach involving the patient and the multidisciplinary team. Staff should partner with the patient/NOK/carer to identify their individual falls risks and frequently discuss falls prevention strategies, especially when there is a change in their condition/environment. The following should be considered in the development of falls prevention care plans and discharge plans:

- The patient and/or carers are made aware of the outcome of the assessment and participate where possible in the planning of interventions, in accord with person-centred care principles.
- The clinical team provides support and care coordination to patients and carers to facilitate their engagement with their care, with consideration of health literacy and cultural and language requirements.
- Staff should consider using interpreters and other communication strategies, as necessary – see the [Interpreter and Translator Guideline](#) for further information.
- Educational messages should aim to address this although some individuals may also need more specialised falls prevention services^{1,17}.
- Written and verbal information can assist but not replace this process. A variety of consumer information is available on the [Safety and Quality information Sharepoint pages](#).

6. Fall prevention and fall injury management

Falls prevention and falls risk factors, are complex and diverse between individuals and their setting. Falls prevention requires an individualised comprehensive and multifactorial approach, and targeted interventions.

6.1 Minimum interventions

All adult inpatients must have the following minimum interventions implemented on admission:

- Provide ongoing orientation for patient to bed area, toilet facilities and ward.
- Demonstrate the use of call bell and ensure it is in reach and that they can use it effectively, and educated regarding Call Don't Fall.
- Ensure frequently used items including mobility aids are within easy reach of patient.
- Encourage patient to use their aids such as glasses or hearing aids. Consider bedside signage.
- Adjust bed and chair to appropriate height for patient.
- Minimise prolonged bed rest as it contributes to negative cardiovascular and muscle effects that may lead to falls.
- Place IV pole and all other devices/attachments on the exit side of bed.
- Remove clutter and obstacles from room.
- Provide adequate lighting according to patient activities/needs.
- Encourage patient to take adequate fluids and nutrition.
- Optimise footwear where possible; discourage walking in socks/compression stockings or ill-fitting footwear. Bare feet (if there is no infection risk) and non-slip socks are acceptable.
- Inform the patient that all inpatients are at increased risk of falling due to injury / illness / medications.

6.2 Individualised interventions

- Patients identified as being at risk of falls require individualised interventions depending on the needs of the patient.
- Additional interventions specific to the patient which are not listed on the FRAMP must be documented on the 'other individualised interventions section' of the FRAMP (in some clinical areas the FRAMP will be on paused for comprehensive care plan), patient care plan, and into the Digital Medical Record via Progress Notes.
- Individual interventions and minimum interventions should be checked each shift and signed off to acknowledge that the interventions have been implemented and/or are still appropriate.

7. Equipment and environmental considerations

As part of a falls management plan, staff should ensure the environment is free of clutter and hazards that may increase the risk of falls. Certain equipment can be used to help manage a patient who is at risk of falls.

Reducing bed moves – for those with a history of falls, it is important that the number of bed moves is kept to an absolute minimum. The number of physical bed moves after the move to the initial admitting ward is significantly associated with experiencing a fall. Each clinical area

has their own falls prevention equipment, refer to local clinical processes. Below is a non-exhaustive list.

7.1 Invisa-beam™ alarm system

Invisa-beam™ (pictured right) is an early warning monitoring device that detects movement to alert staff for falls prevention.

These are primarily held on the Rehabilitation and Aged Care ward, Canning ward, and Anderton Ward (Kalamunda Hospital). Staff are to ensure:

- The alarm must be checked on installation.
- When turning on the Invisabeam™, check the alarm works by triggering it.
- Once triggered, staff must reset the Invisabeam™.
- This safety check is to be performed each time the Invisa-beam™ is turned on and is to be recorded on the patient care plan.
- Ensure cords from Invisa-beam are safe and not a trip hazard.



7.1.1 Bed Alarms

Certain type beds through-out Armadale Kalamunda Group have in built bed alarms. Bed alarms trigger a loud audible sound and activate Nursing Call Bell system when a patient attempts to exit the bed. Liaise with your local Nurse Unit Manager which beds are available.

7.2 Use of an ultra-low bed

When a patient has been deemed a falls risk an appropriate intervention to minimise that risk can be an ultra-low bed. Ultra-low beds are not equipped with bedrails.

Patients that may benefit from the use of an ultra-low bed

There is no single criterion that indicates a patient requires an ultra-low bed. However, consideration needs to be given to the following list which is neither exhaustive nor exclusive:

- Inability to get up off the floor.
- Assessed as being at risk of falls using the Risk Screening & Assessment.
- Impaired mobility (requires assistance or standby with mobility) – needs to be balanced with a low bed impacting on the patient's independence.
- Impaired cognition / confusion e.g. dementia, delirium, scored >4 4AT - needs to be balanced with the possibility that being on an ultra-low bed may increase the patient's confusion.
- Impulsive and attempting to climb out of bed.
- Has sustained a fall during this admission or on previous admissions to hospital.

Where an ultra-low bed is used it is important that this is reviewed each shift as part of daily care - as a patient's condition may change quickly the ultra-low bed may become unsuitable as a strategy to prevent falls for the patient.

- If an ultra-low bed is used it should be placed flush against the wall with no gaps so that if the patient should roll towards the wall, they cannot become entrapped between the bed and the wall.

- Ultra-low beds should not be placed beside equipment that could be pulled down or fall onto the patient, or equipment that could be hit should the patient roll out of bed.
- When the patient is unattended the bed should be lowered to its lower level.

When delivering care, the bed should be raised to a height that suits the staff member delivering that care, and then returned to the lower level when that care has been completed or if the patient is left unattended at any point.

Other considerations:

- Physical illness – some interventions may be difficult or impractical when using an ultra-low bed.
- Psychological illness or distress – the unusual position of the bed may trigger distress, agitation, or increased confusion.
- Poor or lack of sleep.
- Previous accidents or injuries from falls may indicate if an ultra-low bed will reduce the patient's risk of falling.
- Pressure injury - be aware that patients at risk of falls may also be at risk of pressure injury. Some beds are not compatible with some pressure mattresses. Sometimes when a pressure mattress is placed on an ultra-low bed the bed is not then able to be lowered properly to the floor as would happen without a pressure mattress. This results in patients who roll out of bed having a risk of injury. The mattress may also hang over the edge of the bed itself, the pump can be caught under the bed and the cord too short to lower the bed fully. It may be that a patient with a high risk of pressure injury would be better managed on a normal bed at its lowest height. Staff are encouraged to use risk assessment and clinical judgement as well as seek guidance from a senior staff member in this situation.
- Discuss and provide education to the patient/NOK/carer if an ultra-low bed is used then explain the reason it is being used.
- Document the strategies in the Patient Care Plan.
- Include the patient's risk and interventions that are in place during clinical handover.
- Other aspects to consider when a patient is being cared for on an ultra-low bed: Hoists may not fit under the bed readily.
- The ultra-low bed is not to be used for patient transport. Patients must be transferred to a standard bed for transport.
- Ensure that the weight limit of the bed is appropriate for the patient.

To access ultralow beds during office hours the nursing coordinator should page the area Patient Care Assistant. If no beds available the Nurse Unit Manager/Midwifery Manager should be contacted who, if required will arrange hire via (08) 9456 1661 or email info@hospequip.com.au.

7.2.1 Ultra-low beds

The use of ultra-low beds on their own as a falls prevention strategy is not well supported^{15,19}. However, ultra-low beds can reduce the risk of a fall from height, whilst allowing staff to attend to the patient, with consideration to their own back care.

Ultra-low beds must not be a universal falls prevention solution and provided inappropriately to mobile patients. It is important to note that even when ultra-low beds are used correctly in the

lowest position, some patients may still sustain serious injuries such as a fractured hip or intracranial injury¹⁹.

7.3 Bed Rails – General Information

Reducing the use of bedrails does not appear to change the total number of falls that occur in the hospital but can decrease the number of falls resulting in serious injury¹⁵.

Bedrails should never be a substitute for adequate levels of care and observation or used as a stand-alone method of falls prevention¹⁶. Refer to [Safe use of bedrails](#).

7.3.1 Bedrails are not usually appropriate for a patient who:

- Is independently mobile
- Has confusion and is mobile enough to climb over them.
- For patients without decision-making capacity, staff have a duty of care to act in their best interests underpinned by realistic assessment and regular review of the individual risks of bedrail use or non-use.

Special considerations:

- The use of bedrails requires careful, frequent assessment and review.
- When bed rails are in use, the patient call bell, bedside table, and fluids **must be always within reach**.

All use of bedrails plus alternative strategies employed, are to be **clearly documented** in the:

- Patient Care Plan.
- Patient integrated notes or Triage Form (define rationale when used for safety or restraint).
- Behaviour Observation Form as appropriate.

A take 5 is available on the AKG intranet [Safe Bed Rail Take 5](#). The Bedrail Decision Matrix can be found in [Appendix 3](#).

8. Individual surveillance and observation

The following general principles of observation and surveillance represent expert opinion of best practice in the hospital setting, in the absence of trials testing their effectiveness:

- The choice of surveillance and observation approaches will depend on a combination of the findings from the assessment of each patient, clinical reasoning and access to resources and technology.
- An important strategy to consider for improving surveillance is to review staff practices, such as staff handover practices and timing of tea and lunch breaks, to ensure that adequate supervision is available when required.
- Personal preference for the frequency of showers or personal hygiene should be considered on an individual basis and balanced against existing hospital routines²¹.
- Where possible, high-visibility beds or rooms (such as near nurses' stations) should be allocated to patients who require more attention and supervision, including patients who have a high risk of falling²².

Allocation of Assistant in Nursing (AIN) surveillance can occur if the following criteria are met:

- Confirmation of patient's cognitive impairment and associated risks following MDT discussion.
- Patient is impulsive and likely to attempt to mobilise without seeking assistance.
- Patient is likely to fall when attempting to mobilise unsupervised.
- If other strategies for falls prevention have been documented as assessed and deemed insufficient or unsuitable.
- Approval from on-call Management/Executive if after hours – refer to the [AIN – Permanent / Casual / Agency and Booking, Deployment and Supervision Procedure](#).

The Nurse Unit Manager must review the patient's ongoing need for the AIN every 24 hours in collaboration with the MDT. The patient must have a Behaviour Observation Form (AKMR159.1), and the AIN complete the Risk Management Information for Companions and Support Staff.

9. Minimising harm from Falls

9.1 Vitamin D and calcium supplements

- Low vitamin D levels are associated with reduced bone mineral density, high bone turnover and increased risk of hip fracture.²⁸
- For confirmed cases of vitamin D deficiency, supplementation with 3000–5000 IU per day for at least one month is required to replenish body stores²⁹.
- If the patient is having trouble consuming adequate calcium, referral to a dietician may be appropriate.
- People with cognitive impairment are at increased risk of nutritional deficiencies. Staff should monitor patient's oral intake closely via the Malnutrition Screening Tool and refer to a dietician if intake is low.
- Referral to their General Practitioner for completion of Bone Density Screening.

10. Post fall management

Falls and falls-related injuries cause substantial morbidity and mortality among older Australians. For those over 65, falls are a leading cause of injury related hospitalisation in WA¹⁻⁴. Post fall management at any AKG site is to be carried out in accordance with the [Post Fall Multidisciplinary Management Guidelines for WA Health Care Settings](#). The guidelines include the following:

- Discipline specific guidance for nurses/midwives, medical officers, occupational therapists, physiotherapists and pharmacists.
- Nursing guideline and 48-hour post fall process flowchart.
- Medical officer guideline addressing requirements for history taking, physical examination and investigations (including CT head scan rules).
- If a patient with cognitive impairment does fall, reassess their cognitive status, including presence of delirium (via 4AT).
- Next of Kin (NOK) to be notified of fall within 24 hours and documented.

If the patient's condition meets Medical Emergency Team (MET) activation criteria, immediately activate Code Blue / MET call OR if patient has significant physical injuries or on anticoagulant therapy, fast track medical review within 30 minutes of the incident.

Spinal or head injury considerations

Staff should consider the potential risk of spinal or head injury prior to moving the patient:

- Should a spinal injury be suspected senior clinician/medical officer review prior to moving the patient.
- If staff are unable to find the required equipment in the local area, it is essential to the safety of the staff that they wait for the correct assistance and/or mechanical aids.
- Ensure the patient is kept warm and comfortable.

10.1 Revision of the care plan

A re-assessment of fall and fall injury risk will be completed using the FRAMP (AKMR24) or area specific process, within the same shift, and as soon as practicable. Any changes to the care plan must be communicated to the multidisciplinary team, implemented, and documented in DMR.

10.2 Incident reporting

Within the same shift, in-hospital falls are to be reported into the clinical incident management system (Datix CIMS). For consumers of community services, the fall reported is to the General Practitioner. If the fall was related to the health services provided, a risk reassessment including the home environment is performed, the care plan modified; and the fall notified into Datix CIMS.

10.3 Post fall huddle - "Huddle Up"

Huddles are a recognised initiative in healthcare which increase the safety and quality of patient care and systems²⁴⁻²⁵. The Institute for Healthcare Improvement (IHI) and the Clinical Excellence Commission (CEC) support safety huddles as recommended best practice in healthcare settings²⁶⁻²⁷.

Safety huddles improve efficiency, quality of information sharing and accountability. They foster a sense of community and create a culture of collaboration and collegiality that increases collective awareness and capacity for reducing harm²⁴.

It is recommended the post fall huddle process is completed following all inpatient falls. Please refer to [Appendix 4](#) for full details.

Post Fall Huddle:

- Should occur within 24 hours of the incident.
- Should take place at the site of the incident, if feasible, or otherwise at the patient's bedside
- Should include representation from the multidisciplinary team including nursing, medical and allied health staff, where possible

- A member of the multi-disciplinary team should lead and coordinate the huddle discussion, guided by the “HUDDLE-UP” tool
- The patient and/or support person should be included in the huddle
- An interpreter to be arranged if the patient requires one
- All team members are to actively participate and contribute professional expertise to discussion.
- On completion of the huddle, the team should readback the allocated responsibilities for identified team members, including person responsible for actioning referrals to team members not present at huddle.

11. Clinical communication

Information exchange during any clinical handover process must adhere to the [Clinical Handover Procedure](#) requirements using the ISoBAR framework.

11.1 Critical information

Critical information or risks about a patient are communicated in a timely manner to clinicians who can make decisions about the care. The critical information pertinent to this policy includes the following:

- Outcomes of any fall risk assessment should be reported at every clinical handover including implemented and pending prevention strategies.
- Any fall that occurs is handed over to all staff involved in the care of the patient and includes ongoing monitoring, treatment, and prevention requirements.
- Ensure active risks are handed over between the multidisciplinary team via:
 - Bedside shift to shift handover.
 - Ward rounds / huddles.
 - Transfer including transport to/from procedures.
 - MDT meetings.
 - Journey board meetings.
 - Ward transfers.

Handover, regarding falls, to include:

- Patients falls risk and interventions that are in place.
- Recent falls that may have occurred.
- Post falls strategies.

11.2 Documentation

The following documentation requirements must be considered:

- An individualised fall risk assessment and management plan should be documented in the patient's healthcare record as soon as practicable using the approved forms outlined in this policy.
- Update Patient Care Plan to reflect interventions in place to minimise risk of falls.
- Ensure when education about falls is given to patients/NOK/carers that this is documented in the Patient Care Plan.

- On each shift refer to the Patient Care Plan to ensure that falls prevention strategies are in place and review the effectiveness of those strategies.
- Ensure interventions are signed each shift to indicate interventions in place.
- Document interventions in place in the patient integrated notes.
- Include responses to near miss events.
- Document any specific allied health interventions clearly in the Patient Care Plan.
- Reviewed Patient Care Plan each shift and update if needed.
- Review Active Risks Summary and complete a risk re-assessment if indicated, as per Risk Re-Assessment section.
- Post fall medical officer assessment outcomes are to be documented in the DMR Progress Notes contemporaneously and include any medical review, specific observations required and transfer/ escalation parameters.
- A patient's fall risk should be documented into their DMR via Progress Notes.
- Any in-hospital fall incident, including details of the incident, outcome of review and any planned interventions must be documented in the patient's discharge summary by a medical officer.

12. Discharge planning

Where there is ongoing risk of falling, refer to Occupational therapy to assess the need for and eligibility for a home environment safety assessment.

Prior to discharge from hospital, patients with modifiable risk factors still present are referred with appropriate information to appropriate community services for follow-up risk reduction interventions. Services include, but are not limited to, the following:

Service	Description
Allied Health – Outpatient Services	The AKG Referral Database been created to assist local health professionals to enable shared care planning with our local health and social care providers to assist in the management of complex patients with a chronic condition
Rehabilitation in the Home (RiTH)	RiTH provides short to medium term hospital substitution allied health therapy for patients at home. The service aims to facilitate early supported discharge from hospitals or avoidance of hospital admission for patients.
Community Physiotherapy Service (CPS)	CPS provides an out of hospital discharge option for patients requiring further physiotherapy intervention following an episode of inpatient, RiTH, or outpatient care. The service delivers group-based physiotherapy rehabilitation in the community both on land and in water at local community pools and recreations centres.
Community Rehabilitation	Community Rehabilitation is a service where the primary clinical purpose is optimising patient function. A multidisciplinary team provide a coordinated inter-professional patient/carers centred

Service	Description
	service to complex patients with an impairment, activity limitation or participation restriction due a health condition.
Occupational Therapy	Home assessment service for eligible patients discharged from AKG. Referrals need to be from ward OT or if from ED from a member of ED allied health.

13. Roles and responsibilities

Role	Responsibility
Medical Officer	- Review medication - Assess and Manage delirium and depression - Treat osteoporosis - Consider calcium and vitamin D supplements - Document on the Falls Risk Assessment and Management Plan (FRAMP) - Education regarding falls risk and falls prevention
Nurse/Midwife	- Document on the Falls Risk Assessment and Management Plan (FRAMP) or comprehensive care plan - Refer to falls prevention intervention guidelines - Assess for delirium - Orientate to ward and minimise bed changes - Follow nursing care plan for continence and bed rails, keep call bell and mobility aid in reach - Education regarding falls risk and falls prevention
Physiotherapist	- Provide appropriate walking aid, and clearly label - Check bed and chair height - Assess for delirium - Provide appropriate exercise program for balance, strength and mobility - Document on the Falls Risk Assessment and Management Plan (FRAMP) or comprehensive care plan - Education regarding falls risk and falls prevention
Occupational Therapist	- Assess, provide, monitor and review equipment - Encourage patient activity - Assess for delirium and cognitive impairment - Provide individualised cognition, sensory, and environmental strategies - Assess need and eligibility for home safety review - Encourage independence and safe self-care

Role	Responsibility
	- Document on the Falls Risk Assessment and Management Plan (FRAMP) or comprehensive care plan - Education regarding falls risk and falls prevention
Pharmacist	- Identify medications that may predispose patients to falls and review - Where relevant, complete a Fall Risk Assessment via Medication Evaluation (FRAME) tool or comprehensive care plan - Document on the Falls Risk Assessment and Management Plan (FRAMP) or comprehensive care plan - Education regarding falls risk and falls prevention
Patient Support Services	- Return bed to correct height - Place drinks, possessions and call bell within reach - Liaise with nursing for level of assistance required, and bedrails

14. Legislative context

The following documents are required to give effect to this policy:

- HDWA
 - [Clinical Handover Policy MP0095/18](#)
 - [Clinical Incident Management Policy MP0122/19](#)
 - [Recognising and Responding to Acute Deterioration Policy MP0171/22](#)
- East Metropolitan Health Service (EMHS)
 - [Open Disclosure Policy EMHS: 203](#)

15. Supporting information

The following documents support the implementation of this policy:

- HDWA
 - [Post Fall Multidisciplinary Management Guidelines for WA Health Care Settings](#)
 - [Falls prevention and management in WA](#)
- Armadale Kalamunda Group
 - [Interpreter and Translator Guideline](#)
 - [AKG Delirium Clinical Management Guideline](#)
 - [Clinical Handover Procedure](#)
 - [Safe Use of Bedrail Guideline](#)
 - [Open Disclosure Policy](#)
 - [Assistants in Nursing – Permanent / Casual / Agency and Booking, Deployment and Supervision Procedure](#)
 - [Allied Health Provision of Assistive Technology and Home Modifications Procedure](#)

- [Occupational Therapy Issue of Bed Rails for Home Use Guideline](#)
- [Occupational Therapy Home Installations, Fabrications and Modifications Guideline](#)
- Other
 - [SMHS Community Physiotherapy Services - Home](#)

16. Compliance, monitoring, and evaluation

Compliance with the requirements of this policy will be the responsibility of the Clinical Safety and Quality Committee. The minimum requirements for monitoring and evaluation of actions in this policy include:

- Audit of healthcare records to determine whether patients at risk of falling are assessed and managed in line with evidence-based practice guidelines.
- Feedback sought from patients and carers in relation to information provided about falls risk and falls prevention strategies.

The Safety and Quality team will report data and trends of falls and near misses through the departmental Patient Safety Reports, and through reports to the Clinical Safety and Quality Committee. Metrics include counts and rates of falls, and rates of harm from falls (SAC rating and consequences from Datix CIMS), including intracranial injury, hip and other fractures.

17. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards:

- Standard 5 – Action 5.24: The health service organisation providing services to patients at risk of falls has systems that are consistent with best-practice guidelines for:
 - a. Falls prevention
 - b. Minimising harm from falls
 - c. post-fall management.
- Standard 5 – Action 5.26: Clinicians providing care to patients at risk of falls provide patients, carers and families with information about reducing falls risks and falls prevention strategies.
- Standard 6 – Action 6.9: Clinicians and multidisciplinary teams use clinical communication processes to effectively communicate critical information, alerts, and risks, in a timely way, when they emerge or change to:
 - a. Clinicians who can make decisions about care.
 - b. Patients, carers, and families, in accordance with the wishes of the patient.

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19. Glossary

The following definitions are relevant to this policy.

Term	Definition
Fall	An event which results in a person coming to rest inadvertently on the ground or floor or other lower level. If a consumer is found on the floor or lower level, it should be assumed that a fall has occurred unless there is reasonable evidence of a sudden onset of paralysis, epileptic seizure, and loss of consciousness or overwhelming external force (being pushed).
Fall risk assessment	A detailed assessment that identifies the individual's risk factors for falling and for fall related injury. This will assist with clinical decision-making by indicating which interventions should be included in the care plan.
Fall risk screen	A brief test that gives an indication of the patient's overall level of risk of falling. This may indicate the need for more detailed assessment.

20. Policy owner

Director Allied Health and Ambulatory Care

Enquiries relating to this policy may be directed to:

Title: A/ Head of Department Physiotherapy
 Department: Physiotherapy
 Email: Christopher.Owens@health.wa.gov.au

21. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	10 12 2025	10 12 2028	Comprehensive scheduled review, significant updates throughout document.

22. Authorisation

This policy has been authorised and issued by:

Authorised by	Danielle Kilmurray – Director Allied Health & Ambulatory Care
Authorisation date	09 12 2025
Published date	10 12 2025

23. Appendices

23.1 Appendix 1: Fall risk screening, assessment, and care

RISK	FACTOR/S	INTERVENTION/S
Patient Information	- Reason for admission (e.g. # NOF, fall) - Relevant past medical history (e.g. stroke, # NOF) - Are they on Medication/s increasing risk of falls? (e.g. sedatives, antihypertensive, antipsychotic)	Monitor postural BP before mobilisation Educate patient regarding falls risk and interventions / refer to the Safe Recovery Program (if available)

	Is their age (>65 years or >45 Aboriginal people)	
Communication	- Hearing impairment - Visual impairment - Speech impairment / English as second language	- Encourage use of aids /assist as needed - Appropriate environment / decrease clutter - Utilise cue cards / utilise interpreter
Social	- Do they have carer and why? - Do they have any modifications to their home to help mobilise?	Appropriate level of assistance where needed
Pre-Admission Status	- How did they function before coming into hospital? - Aids to assist ADL's - Glasses? - Hearing aids? - Dentures?	- Required aids are available - Glasses used? With the patient? - Hearing aids used? With the patient?
Infection Prevention & Management	- Do they have any invasive devices that restricts movement or presents falls hazard? - Will they be in an isolation room with limited visibility compared to a shared room?	- Ensure all lines/device secure - Educate patient of potential hazard - Ensure call bell in reach and patient educated - Move pt to higher visibility area / companion - Place devices/attachments on exit side of bed
Swallowing	Is the patient being sat out of bed to eat meals?	- Support patient in transfer to chair - Appropriate chair in use / adjusted for patient
Bariatric Status	- Do they use aids? - Do they have restricted independent movement?	- Specialised bariatric aids - Specific assistance with ADL's as required
Nutrition	- Are they malnourished? - Weak muscles / muscle atrophy	- Dietitian review - Dietary Intake Chart - Appropriate level of assistance where needed
Skin Assessment	- Do they have any wounds that affect their mobility? - Certain dressings and location can affect mobility - Is pain restricting their mobility? - Altered Sensation - Any loss of sensation may inhibit their mobility e.g. peripheral neuropathy	- Appropriate footwear - Required aids as needed - Appropriate level of assistance where needed - Encourage mobility - Appropriate pain management
Elimination	- Level of assistance with toileting - Degree of constipation, urinary or faecal frequency/continence/ urgency/ nocturia - Access to toilet facilities - Effects of medication/treatment	- Required aids in reach / location of toilet - Provide an appropriate toileting plan (NOT prn) - Level of assistance when needed - Appropriate continence pad in situ
Mobility	- Previous falls (within last 12 months)? - Able to turn self in bed? - Unsteady when walking or when transferring? - Feeling dizzy / light-headed? Postural hypotension? - Fear of falling? - Is pain management adequate? - Does patient have impairment of balance?	- Physiotherapist review - Postural BP - Mobility aids in reach - Removal of clutter and hazards - Provide level of assistance required - Ensure the patient's chair meets their needs
Cognition	- Head trauma in last 48hrs? - Positive to 4AT / SQID - Impaired cognitive state or known cognitive impairment - Unable to follow instructions - New environment - Impulsive	- Utilise ultra-low bed / bed against wall - Mobility aids in reach & appropriate footwear - Bed rails – utilise only if appropriate - Removal of clutter and hazards - Orientation board / tools

		• Involve family in care / refer to Forget Me Not • Refer to MDT for further cognitive assessment
Behaviour	• Impulsivity / decreased insight • Refusing care • Restraints required (pharmacological or mechanical)	• Mobility aids in reach • Removal of clutter and hazards • Increase supervision/involve carer • Consider use of fall alarms • Consider reducing stimulation
Drugs/Alcohol/ Nicotine	• Are they currently intoxicated? • Disorientated / Dizzy • Are they withdrawing? • Tremors – unsteady • Sweating – slippery • Altered mental state	• Appropriate footwear • Orientation board • Appropriate mobility aids in reach • Consider NRT/referral to specialist service • Monitor utilising withdrawal charts

23.2 Appendix 2: Falls Risk Assessment by Medical Evaluation (FRAME) Tool



EMR307790

Please use I.D. Label or BLOCK PRINT

ARMADALE KALAMUNDA GROUP FALLS RISK ASSESSMENT BY MEDICATION EVALUATION (FRAME) TOOL	Family Name: _____ Given Names: _____ Gender: _____ UMRN: _____ DOB: _____
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List in the table any medication(s) that patient is taking from the groups below:

Medication groups	Initial assessment Date _____	2nd assessment Date _____
Benzodiazepines / hypnotics		
Antidepressants		
Antipsychotics		
General anaesthetics / pre-medication		
Anti-angina medications		
Opioids		
Medications for epilepsy		
Parkinsonian medications		
Anti-hypertensives		
Diuretics		
Anticholinergics / drugs with anticholinergic side effects		
Dementia medications		
Aperients / laxatives		

Document suggested falls minimisation strategies and outcomes below:

Date / Time / Sign	Suggested Strategy (intervention)	Discussed with (list name)	Outcome

A255
12/22

AKMR24.2 FALLS RISK ASSESSMENT BY MEDICATION EVALUATION (FRAME) TOOL

The FRAME tool (AKMR 24.2) facilitates a multidisciplinary approach to medication reviews, with the aim of optimising pharmacotherapy to minimise falls risk.

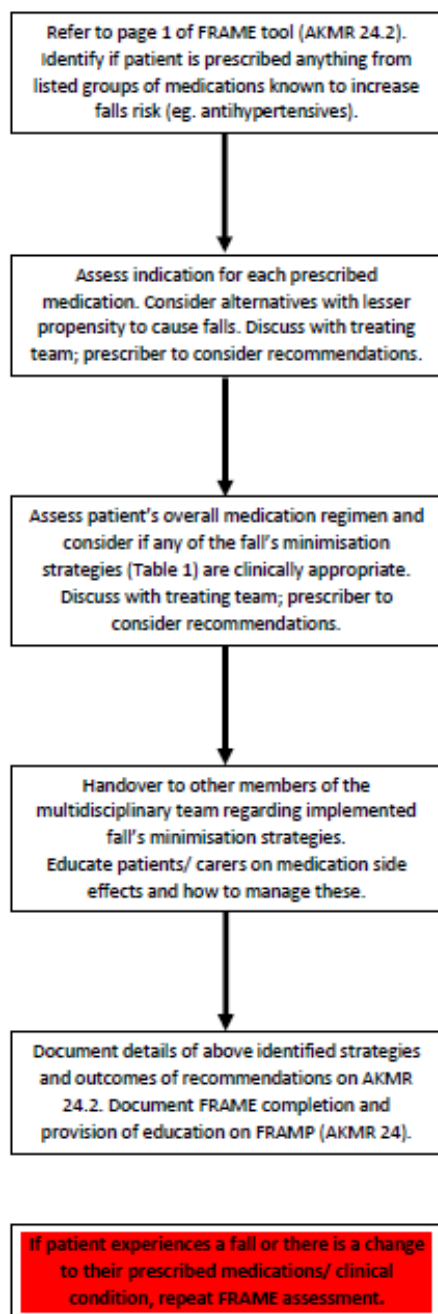


Table 1 Falls Minimisation Strategies (consider implementing the below, where appropriate)

Strategy	Suggested intervention
Consider alternative to sedatives	- Sleep hygiene strategies to be used first line for managing insomnia - Consider melatonin in place of benzodiazepines
Identify and rectify other causes of insomnia (to minimise need for sedatives)	- Ensure medicating timing is appropriate (eg. those causing insomnia best prescribed for morning administration, if possible) - Review ongoing need for concurrently prescribed medications that may cause insomnia.
Assess adverse effect potential	- Select medications with more favourable side effects, if possible (eg. nortriptyline over other tricyclic antidepressants due to lesser anticholinergic potential) - Selecting medications that are more favourable to patient's disease state (eg. quetiapine preferred in Parkinson's Disease)
Assess pharmacokinetics	Prescribing medications with a shorter half-life (eg. oxazepam over lorazepam) in the elderly to minimise prolonged sedation
Medication rationalisation	Review ongoing need for prescribed medications that have no obvious indication
Dose review	- Where appropriate, low starting doses should be initiated and titrated accordingly - Assess patient's medication compliance prior to hospital admission. May need to consider restarting at a lower dose and re-titrating.
Set parameters for medication administration	Determine parameters for when appropriate and inappropriate to administer medication (eg. "withhold antihypertensive if blood pressure below a certain range")
Review medication timing	- Consider pharmacokinetics of prescribed medication and review if prescribed time is appropriate (eg. diuretics should ideally be prescribed no later than 2pm, unless patient is catheterised) - Provide education to staff and patients/ carers about expected onset of action of laxatives

Special considerations:

Cognitive impairment:

- Consider dosage administration aids to assist with medication compliance. This may be implemented as a trial in hospital.
- Where appropriate, prescribers should aim to simplify prescribed medication regimen (eg. if possible, prescribe a medication that is administered once a day instead of three times a day).
- It can be challenging for patients with cognitive impairment to communicate issues, making subjective assessments unreliable. Special attention needs to be given to altered behaviours and non-verbal cues.

23.3 Appendix 3: Bedrail Decision Matrix

Decision-making capacity	Level of mobility			
	Cognitive/mental state	Unable to mobilise or hoist-dependent	Requires assistance to mobilise	Independent mobility
No	Delirious, confused, disoriented, agitated, restless, unpredictable poor memory.	Consider bedrails with caution	Bedrails not recommended. Use alternative strategies.	Bedrails not recommended. Use alternative strategies.
Uncertain	Drowsy/sedated/ impaired consciousness.	Consider bedrails with caution	Try alternative strategies, and only use bed rails as last resort. Frequent monitoring required.	Try alternative strategies, and only use bed rails as last resort. Frequent monitoring required.
Yes	Orientated and alert.	Bedrails may be considered if the patient consents or requests them.	Bedrails may be considered if consumer consents or requests them. Ensure they can summon help.	Bedrails not required. Can be requested by consumer.

23.4 Appendix 4 - Post Fall Huddle

H Hello & huddle up	<i>Introduction, explain purpose of huddle</i> Acknowledge and introduce each person who is present and explain their role. Explain purpose of the huddle to patient, which is to try and prevent recurrence of a similar incident.
U Understand what happened	<i>Document Incident</i> Any staff members involved should provide information of the event. Staff members must be de-identified in documentation: refer to position title, not to person's name. Patient/ family member should be given the opportunity to describe the incident from their perspective Outline any harm/injuries the patient sustained.
D Open Disclosure	<i>Offer an apology</i> Shift coordinator / NUM offers an apology to the patient and/or family on behalf of the ward/service for the incident that occurred. Inform the patient/family that incident will be recorded into the Incident Management System. <i>Outline what actions have been taken by team since fall e.g. observations, investigations</i>
D Discuss what contributed to the event	<i>Discuss contributing factors</i> Examples may include: items inaccessible, wet floor, communication barriers. Review the pre-incident documentation to identify gaps in prevention and management of risks.
L List the risk factors	<i>Review and list risk factors</i> Examples may be: postural hypotension, reduced mobility, cognitive impairment, visual impairment, delirium/dementia, incontinence, sensory impairment, medical devices, equipment.
E Eliminate or manage risk factors	<i>What will help manage/reduce the risk?</i> Identify and outline specific strategies. For further support: Refer to AKG Fall and Fall Injury Prevention and Management Policy
U Undertake actions and allocate responsibilities	<i>What do we need to do?</i> Identify actions required and who is responsible for each task to ensure the actions are followed up. Provide education to the patient (ensure interpreter is used where required) Involve and collaborate with patient/ family when developing the plan, where possible. Handover of all the recommendations provided to nursing, medical and allied health staff caring for patient
P Plan	<i>What have we learned from this event?</i> <i>What will change because of this incident?</i> Consider sharing as a case study at team meetings.